A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

13 May 2026

A/Prof Michael Hii
Suite 301, Level 3

Dear A/Prof Hii,

Re: Ms Demi Lampropoulos
DOB: 16/05/1999 Phone: 0422 309 888
Address: 19 Staples Way, SEABROOK, VIC, 3028
Email: demi.lampropoulos.16@gmail.com

Summary

- **Review of barium swallow, oesophageal manometry, and biopsies showing no evidence of a primary oesophageal motility disorder.**
- **Escalation of amitriptyline to 50 mg for suspected oesophageal hypersensitivity.**
- **No action required by you.**

I had the pleasure of conducting a review with Demi to discuss her ongoing symptoms and the results of her recent investigations. Since our previous consultation, I have reviewed her barium swallow, manometry, and histology. Reassuringly, there is no evidence of a major oesophageal muscle or motility disorder, and her biopsies did not reveal any significant structural pathology.

We discussed that her symptoms likely represent a degree of oesophageal hypersensitivity or a functional coordination issue. While she has previously trialled budesonide and baclofen without significant benefit, she continues to use biotin and a throat spray for transient relief of her dry mouth and throat discomfort. She is currently taking 20 mg of amitriptyline for headache management, as well as Keppra.

Impression and Plan

My impression is that Demi's symptoms may respond to further neuromodulation to alter oesophageal nerve sensitivity and muscle function. We discussed several therapeutic avenues, including the use of calcium channel blockers or prokinetic agents like prucalopride, but given she is already tolerating amitriptyline, I believe the most logical next step is a strategic dose escalation.

I have provided Demi with a prescription for 25 mg amitriptyline tablets. I have asked her to increase her dose slowly from the current 20 mg up to 25 mg, then 37.5 mg, reaching a target dose of 50 mg as tolerated. We will allow six to eight weeks at this dose to properly gauge the clinical response. We also touched upon her post-nasal drip and dry mouth; however, I would prefer to focus on the amitriptyline titration before introducing further antihistamines or medications.

Finally, we addressed her concerns regarding her known intracranial lesion. I have reassured her that this is unlikely to be the cause of her swallowing symptoms, as the primary swallowing centres are located deeper within the brainstem. I will review her in approximately ten weeks to assess the efficacy of this higher dose.

All appointments and enquires
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Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

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Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Sumbla Khalid

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